

RIVERSIDE MULTISPECIALTY HOSPITAL

Department of Endocrinology

Follow-up Consultation Note

Patient Name:	John Doe	Medical Record #:	MRN-2026-001247
Age / Sex:	54 years / Male	Date of Visit:	2026-07-20
Consultant:	Dr. Sarah Smith, MD (Endocrinology)		

REASON FOR FOLLOW-UP

Routine follow-up for management of Type 2 Diabetes Mellitus following lifestyle modification and Metformin therapy.

INTERVAL HISTORY

Mr. John Doe reports continued adherence to Metformin and remains committed to dietary modifications. He continues walking approximately 30–40 minutes on most weekdays. He denies excessive thirst, frequent urination, blurred vision, dizziness, chest pain, palpitations, or shortness of breath.

However, over the past month he has experienced intermittent mild headaches, particularly during work hours. He attributes this to increased workload and stress but denies any visual disturbances or neurological symptoms. Home glucose monitoring continues to demonstrate satisfactory glycemic control.

CURRENT MEDICATIONS

Medication	Dose	Status
Metformin	500 mg orally twice daily	Continue
Amlodipine	5 mg orally once daily	New Prescription

PHYSICAL EXAMINATION & VITALS

Parameter	Current Visit	Previous Visit
Blood Pressure	150/92 mmHg Repeat (10 min): 148/90 mmHg	132/84 mmHg
Heart Rate	78 bpm	76 bpm
Respiratory Rate	16 / min	16 / min
Temperature	98.3 °F	98.2 °F
Weight	87 kg (-1 kg)	88 kg
Height	173 cm	173 cm
BMI	29.1 kg/m²	29.4 kg/m²

LABORATORY RESULTS

Test Description	Current Result	Previous Result
HbA1c	7.3%	7.5%
Fasting Plasma Glucose	118 mg/dL	124 mg/dL
Random Blood Glucose	160 mg/dL	168 mg/dL
Serum Creatinine	0.9 mg/dL	0.9 mg/dL
eGFR	>90 mL/min/1.73m ²	>90 mL/min/1.73m ²
Total Cholesterol	204 mg/dL	Not Previously Tested
LDL Cholesterol	132 mg/dL	Not Previously Tested

ASSESSMENT

- **Type 2 Diabetes Mellitus:** Remains under satisfactory glycemic control with continued gradual improvement in HbA1c (7.3%).
- **Essential Hypertension (New Diagnosis):** Persistent elevated blood pressure readings on multiple measurements (150/92 mmHg), matching the clinical window of work-hour headaches.
- **Mild Hypercholesterolemia:** Noted on routine lipid profile (LDL 132 mg/dL).
- **Renal Function:** Remains completely stable and preserved.
- **Adherence:** Patient remains highly motivated and compliant with medical/lifestyle treatments.

TREATMENT PLAN

- **Anti-Hyperglycemic:** Continue Metformin 500 mg orally twice daily.
- **Anti-Hypertensive:** Initiate Amlodipine 5 mg orally once daily.
- **Cardiovascular Care:** Recommend home blood pressure monitoring; advise strict reduction of dietary sodium intake.
- **Lifestyle:** Continue current diabetic diet, weight management, and regular exercise regimen.
- **Follow-up Evaluation:** Repeat blood pressure evaluation in 8 weeks; repeat HbA1c in 3 months.

PATIENT EDUCATION

The patient was educated regarding hypertension and its increased cardiovascular risk in individuals with diabetes. Lifestyle modifications including weight management, exercise, stress reduction, and sodium restriction were discussed. The patient verbalized understanding and agreed with the proposed management plan.

FOLLOW-UP

Return in 8 weeks for blood pressure review and medication assessment. Continue regular diabetes follow-up thereafter.

Electronically Signed By:
Dr. Sarah Smith, MD
Consultant Endocrinologist
Riverside Multispecialty Hospital